

HITH-Hospital in The Home

OPERATIONAL TEAM MEETING

DATE: 03.09.2025
TIME: 14:30-15:30
VENUE: MS TEAMS
CHAIR: RACHELLE FOXTON
MINUTES: MARZIA SALVALAGGIO

MINUTES

Ref #	Description:	Presenter
1	Acknowledgment of Country: I would like to acknowledge the Traditional Custodians of the Northern Sydney Local Health District region, the Dharug and Guringai peoples. Their spirit can be found across the land, and we honour the memory of their ancestors and Elders, past and present. As we endeavour to serve the health needs within the community, we recognise the importance of land and waterways as an integral part of people`s health and wellbeing.	Rachelle F.
2	Attendance: 03.09.2025 HITH OTM Attendance.xlsx Apologies: Kate Lock (A/L), Jane Yu (A/L), Maureen Sutton (LSL), Samantha Chan.	Rachelle F.
3	Confirmation of Meeting Minutes: Therese Coleman (NUM3) confirmed the minutes from the 5th of August.	Therese C.
4	Patient Story/ Example of Person-Centered Care: Dr Michelle Chen shared an email from the Patient Experience team about a patient who suffered severe breathing difficulties, diagnosed with influenza A on top of COPD, and received exceptional support from medical and nursing staff in the emergency and respiratory wards, as well as from HITH & VCS. The patient expressed profound gratitude for the high-quality care. Michelle Chen highlighted the significance of this positive feedback, noting it came from a respiratory physician who typically offers criticism. The patient also commented on the exceptional job received, particularly over the winter period.	Michelle C.
5	Staff Achievement/ Staff Compliments: Nil discussed. Apologies as Shelley was unable to present it.	Shelley S.
6	New Business: - <u>HealthDirect</u>: The feedback received from the new platform, HealthDirect, which has replaced My Virtual Care. Bruno (Virtual Care Manager) suggests that clinicians actively remind patients about the post-consultation survey. This aims to increase survey completion, gather valuable anonymous feedback on patient experience (including technical issues), and foster a two-way dialogue to continuously improve practice. - <u>Patient Safety & Culture Survey feedback</u>: Rachelle proudly reinforced the team's strong commitment to safety and quality in all interactions with patients, carers, and each other, emphasising mutual support in delivering safe care. Shelley already shared the excellent performance result via email. - <u>Ways of working</u>: The management team acknowledged staff's hard work during a busy winter and supported each other. - <u>Teamwork & Support</u>: Nurses with lighter patient lists should proactively assist busy colleagues and promptly notify coordinators of patient changes such as readmission or patient refusal to come to the service.	Shelley S. Rachelle F. Amy H. Therese C.

	- <u>Evening Shift Management</u>: Staff are encouraged to start visiting patients early in the afternoon to allow flexibility for unexpected calls later, and to use the full shift until 22:30 for work or professional development, such as completing MHL modules, reading emails and watching in-services, if patient care is completed. - <u>Vehicle Policy</u>: Clarified that taking work cars home overnight is not expected unless essential for facilitating early morning visits. Staff should otherwise use personal transport to maintain workflow flexibility for coordinators. The MN shift is a 'floater' shift, and there is reduced flexibility if cars have been taken home and are not available for any changes with patient loads. - <u>Meetings and in-services</u>: The staff is kindly requested to turn on webcams during online meetings and education sessions to show engagement and respect for presenters. We have made every effort to ensure that all workstations have camera access, whether through a built-in screen camera, a laptop camera, or a separate webcam. If you are experiencing any issues with your camera, please notify your manager. If necessary, submit a SARA request to resolve the issue. - <u>People Matter Employees Survey (PMES)</u> – Due now. You can provide your feedback through the survey by accessing the following link: https://www.health.nsw.gov.au/peoplesmatterssurvey - Rachelle encourages the team to exceed last year's performance and offers an opportunity for employees to provide feedback on areas for improvement. Employees can use this designated feedback channel or speak directly with the management team, as they are always reachable.	Rachelle F.
7	Items for Notation / Information / Correspondence: - A safety alert was issued regarding high influenza cases in paediatrics – encouraging vaccinations. Please see the tabled documents below. - Michelle also highlighted awareness regarding a prolonged influenza (B strain) outbreak. Key points include maintaining a low threshold for Tamiflu in immunocompromised and other vulnerable patients and emphasising that it is not too late to get vaccinated. They highlight serious outcomes, particularly in children (which can also affect elderly caregivers), and warn of severe complications like encephalitis and myocarditis.  FW SA005 25 Influenza in children Recognising, Preventing and Responding to Complications .eml  SA005 25 Influenza in Children Recognising Preventing and Responding to Complications.pdf  SAFETY NOTICE 005 25 Influenza in childrenSAAP.doc	Shelley S. Michelle C.
8	Minutes from Advisory Committees:  AH Steering Committee Referral Note August 2025 Endorsed.pdf	Rachelle F.
9	Standing Agenda Items: 9.1 Resignations, Vacancies, Recruitment – Allied Health, NUMs: NB NUM: <u>Resignations:</u> - After the resignations of Stephanie and Olivia, we currently have several open RN positions. These include both temporary and permanent roles, contract, part-time and full-time shifts available. Therese and the team would like to extend our deepest gratitude to Steph, who is retiring after an incredible 25 years with HITH. Steph has been an amazing nurse with exceptional clinical skills, and her contributions have been invaluable. We wish her all the best as she begins her new journey and focuses on her artwork. Steph has requested a farewell party, and we will be organising a card and money collection for her soon. - We are also saddened to say goodbye to Olivia, who has resigned to spend more time with her family. Olivia's positive and can-do attitude has made a significant contribution to our team. She shared that she is sad to	Therese C.

	leave but feels this is the right decision for her family, and that our team was the best with which she has ever worked. <u>Recruitment:</u> - Please join us in giving a warm welcome to Clare Collins, our newest RN, starting on Monday September 15th, and will be working 0.5 FTE. She is an experienced nurse and educator, joining us from the Mater Hospital, where she worked for 17 years. We are excited to have her as part of the team at Mona Vale Hospital. NS NUM: <u>Resignations:</u> - Sibbo has resigned from her RN position to accept a permanent part-time CNC role with Northern Sydney, marking a significant career advancement after 18 months in a secondment. Sibbo, who was with her previous employer for ten years, will continue working one day a week in the intake role on a temporary contract until January. Her departure creates a full-time RN vacancy that will soon be advertised. <u>Recruitment:</u> - We have some temporary and part-time/ vacancies that are currently filled with casual staff. Thanks to all the staff who are currently helping within the organisation. AHM: <u>Resignations:</u> Nil <u>Recruitment:</u> - Rachelle announced three new team members. Carol Cheng, a Social Worker who has joined the North Shore team, works Tuesdays and Wednesdays. An Occupational Therapist covering Thursdays and Fridays is expected to start shortly, date to be confirmed (acknowledging Anj as our sole OT for the next 3 weeks due to a vacancy and planned leave). A Physiotherapist will join from October 6th (weekends and late week). These additions aim to enhance team support and cover staffing gaps. 9.2 HITH/VCS Medical Director Update: Michelle Chen - The new Aged Care HITH and VCS Doctors, which launched on July 1st, are operating well. It uses existing medical staff (David, Sina, Keat, Lynn, and Kieran) to provide care. Teams are currently working to resolve initial issues with handovers, as well as business rules regarding results follow-up and handbacks from BRACE/GRACE/ARRT - Discussions are underway to reduce the weekend workload by minimising unnecessary blood tests and radiology requests. The goal is to clarify who is responsible for follow-up care for these procedures. - <i>GP Initiatives:</i> ➤ <u>Iron Infusions:</u> As of October 31st, routine iron infusions will no longer be offered as a routine treatment, rather as a case-by-case basis in addition to planned HITH treatment. This should create more capacity. ➤ <u>Diverticular Pathway:</u> A new pathway for managing uncomplicated diverticular cases, originating from Hornsby, was implemented on September 1st. Engagement with general practitioners (GPs) is ongoing to support this change. - Efforts are being made to improve the management of complex elderly patients referred by GPs. This includes a focus on enhancing handovers, ensuring clear follow-up plans, and defining endpoints for their care to address the significant challenges these cases present. 9.3 HITH Nurse Manager Update: Shelley Somi	Amy H. Rachelle F. Michelle C. Shelley S.
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- <u><i>Patient Safety Culture Survey</i></u>: A big congratulations to the entire HITH team! We achieved a remarkably high response rate, which is a fantastic result. Because of this outstanding participation, we have been awarded a \$1,000 prize. This money can be used for resources, staff education, or team wellbeing. We want your input on how to spend it! Please let your managers or me know if you have any ideas. We also ranked in the top quartile or decile for all metrics in the survey. The scorecard will be sent out to the team soon, and we will work on an action plan to further improve in any areas identified. - <u><i>People Matters Survey</i></u>: Staff are encouraged to complete it to support staff satisfaction and retention; coffee vouchers are offered as an incentive. Staff feedback is valued as much as we focus on our patient surveys – please complete if you have not already. Link is in the chat to support completion. - <u><i>Iron Infusions</i></u>: Direct GP referrals for iron infusions will cease from the end of October to refocus on core business and increase capacity, though occasional secondary use may continue. A memo has been sent to GPs. Refocus on HITH core business with hospital substitution or ED avoidance. - <u><i>Diverticulitis Pathway</i></u>: Commenced on the 1st of September, with low initial referral rates anticipated. No referrals yet. Approximately anticipating 1-2x referrals per week. <u><i>Documentation</i></u>: A reminder to thoroughly document in EMR, not MS Teams, as Teams is not part of the official medical record. There is a plan to circulate the revised business rules for Teams shortly.	
9.4 Pharmacy: Horst Thiele/Therese Cameron - <i>After-Hours HITH Pharmacy Guide created by Therese with thanks.</i>: Is now available on the front page of the HITH resource page in SharePoint. - <i>ACOS Referrals</i>: The HITH RMO now files the HITH Intake Notes for Aged Care Outreach Services (ACOS) referrals will now include patient resource requirements for quick reference, especially in cases of rapid deterioration or resus requirements. - <i>Patient Falls</i>: A reminder to report patient falls on service (as per the HIS falls brochure) so that medications can be reviewed for potential contribution. - Please see the tabled documents regarding RNSH Medication Matters for July and August, as well as the RNSH Bi-Monthly Dose Newsletters. The July-August Bi-Monthly Dose.pdf - 25-07 Medication Matters - Duplication of Therapy.pdf 25-08 Medication Matters - Medication Administration Alerts.pdf	Horst T.
9.5 CNC Update: Michelle Lane - <i>Diverticulitis Referrals Launch</i>: The referral program started on the first of September. Staff are encouraged to review the process on SharePoint and watch presentations by the dietitian (Justin) and on the diverticulitis pathway. Registered Nurses specifically need to take out observation equipment (locations emailed) and patient information envelopes (containing welcome letters, diet info, etc.) on the first day when a patient is allocated. - <i>Skin Tear Prevention</i>: Following a quality report, new blue silicone tape is now available to prevent skin tears, particularly for patients with fragile skin, those on warfarin, or who are immunocompromised.	Michelle L.
9.6 NP Update: Liana Scott - <i>The CADD Pump project</i> is progressing very well, with the tenth patient having just started. North Shore staff are managing it effectively, and specific training is planned for Hornsby to boost their involvement. No significant issues have been reported.	Liana S.

9.7 Education: Samantha Chan - CNE Kate Lock is currently on AL until 08.09.25. Sam is available. - We had five in-services via Teams in August, including Administration of Electrolytes, Pathology Results interpretation by Dr Virginia. Diverticulitis pathway & the diets and Wound escalation. Recaps are available on Teams or in the HITH resources site. Please ask your CNEs for any help. - We had one Lifepak 20 Defib in-person training session for HKH staff, which was completed on 26.8.25. More training sessions are to be organised for other HKH staff. - Annual Manual Handling skills reviews are ongoing, thanks to those of you who have already completed this. - The PACH orientation is on 16,17, and 23 of September.	Sam C.
9.8 Allied Health: Rachelle Foxton - Thank you for supporting and triaging urgent/non urgent patients whilst we have had planned and unplanned leave in the allied health team.	Rachelle F.
9.9 GP Liaison Update: David Lewis The GP liaison has primarily been managing out-of-area hospital liaison work, including covering annual leave in the Northern Beaches Hospital. - David would like to remind the staff to always ensure laptops are fully functional, especially cellular connection, before visits to avoid issues; contacting support if needed (1300 28 55 33). Check by unplugging from the docking station before going on your visit to test the Cellular function and ensure you continue to have access to EMR and Teams before departing. - Thank you for prioritising and confirming the current patient's phone number and requesting that an update be completed with the community front sheets, noting that contact details might be outdated. - The GP liaison will assist with education on new Iron Deficiency Anaemia referral processes for GPs and manage related stakeholder feedback. Any urgent issues should be escalated to Shelley, Coordinators, or the GP liaison for a coordinated response.	David L.
9.10 Admin Update: Neelam Sood Neelam highlighted the significant growth and achievements of the service in July. Some key points include: - A record of 297 patient separations occurred in July 2025, the highest in four years, reflecting increased team activity. - Overall service activity increased by 11% from last July, with NSLHD hospitals seeing a 38% rise. - Direct referrals: The team is already exceeding a future KPI from direct referrals, achieving 58% in July against a 50% target set for July 2025 (exceeding the target) - Clinic data have shown that July was the busiest month for Hornsby and North Shore since March 2025 - In July, we had 208 virtual care events provided, primarily via phone, with video consultations increasing due to Health Direct. 16% audiovisual consultations - Amy, on behalf of the front desk team, would like to request that staff provide patients with NSHNS's direct contact details when making referrals. This is to prevent confusion, as patients are currently contacting their facility for post-discharge wound care and visits, mistakenly believing them to be Sydney Home Nursing. A suggestion was made to create small cards with these details for patients that the Front Desk could create and have available for staff to supply to patients to facilitate this handover.	Neelam S. Amy H.

10	WH&S General Updates: 10.1 Hazard Registrar: - <i>North Shore:</i> The Douglas Building ramp issue has been escalated, awaiting a response and action. - <i>Mona Vale:</i> Air conditioning repairs are in progress, with marine-grade chillers being installed. - All Staff are encouraged to report hazards to the NUM 1 or your manager, though no new concerns have arisen beyond routine monthly inspections. - <i>WorkSafeGuardian:</i> The system is successfully adopted with thirty-seven registered devices, and staff are commended for their effective use and district leadership. - <i>Monthly Inspections:</i> A reminder for staff to complete August and September inspections. <table><tr><th>Month</th><th>Beaches</th><th>Completed</th><th>RNS</th><th>Completed</th></tr><tr><td>August</td><td>Tamami & Tony</td><td></td><td>Louise & Gayle</td><td></td></tr><tr><td>September</td><td>Therese & Jess</td><td></td><td>Maureen(A/L) & Carol</td><td></td></tr><tr><td>October</td><td>Jayne & Clare</td><td></td><td>Amy & Rachelle</td><td></td></tr></table> 10.2 Manual Handling: Annual Manual Handling Training (annual skills review) continues to be planned, and the staff’s active participation is appreciated. 10.3 Security: No recent updates 10.4 Code Purple, Black, and Red Tabletops: Planning for Hornsby and Mona Vale is ongoing, with dates to be announced. <table><tr><th>EXERCISES</th><th>RNSH</th><th>BEACHES</th><th>HORNSBY</th></tr><tr><td>Code Red</td><td>04th June 25</td><td>TBC</td><td>TBC</td></tr><tr><td>Code Black</td><td>07th April 25</td><td>27th May 25</td><td>21st May 25</td></tr><tr><td>Code Purple</td><td>08th April 25</td><td>27th May 25</td><td>21st May 25</td></tr><tr><td>Bushfire</td><td>Sept 25</td><td>Sept 25</td><td>Sept 25</td></tr></table> 10.5 Wellness: Encourage staff to look at this SharePoint and explore options available, such as Fitness Passport. Please see the SharePoint link: Health and Wellbeing	Month	Beaches	Completed	RNS	Completed	August	Tamami & Tony		Louise & Gayle		September	Therese & Jess		Maureen(A/L) & Carol		October	Jayne & Clare		Amy & Rachelle		EXERCISES	RNSH	BEACHES	HORNSBY	Code Red	04 th June 25	TBC	TBC	Code Black	07 th April 25	27 th May 25	21 st May 25	Code Purple	08 th April 25	27 th May 25	21 st May 25	Bushfire	Sept 25	Sept 25	Sept 25	Rachelle F.
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11	Multicultural Health and Aboriginal/Torres Strait Islander Updates: The proposed language changes for the REACH brochures are currently on hold due to internal changes within the Ministry; they are awaiting a final decision before proceeding.	Heather T.																																								
12	Sustainability: Nil	All																																								

13	Business Without Notice: - Corey requested the team to limit non-essential messages in Teams chats, especially during handovers, because flooded chats make it difficult to find important patient care information among various messages. Chats are for planning and coordination with CC. Please reduce the ‘noise’ that is unnecessary information or not patient related. Thank you. - Therese shared the information that the extended EMR (Electronic Medical Records) downtime is scheduled for October 14th, from Tuesday afternoon through Wednesday morning. This will impact all departments, requiring managers to develop department-specific action plans for preparation. - Teresa would like to remind that daily clinicians are requested to contact patients between 8:00 and 10:30 AM to provide an approximate visit time (ETA). While acknowledging that circumstances like unanswered calls may prevent this, it is emphasised as important for both patients and clinicians to plan their day effectively.	Corey L. Therese C. Teresa L.
14	Date, Time & Venue for the Next Meeting: Tuesday, 7 th of October 2025 with Therese Coleman	Rachelle F.
15	Meeting Closed: 15:31	Rachelle F.

2025												
Jan	Feb	Mar	Apr	May	Jun	Jul	Aug	Sep	Oct	Nov	Dec	Jan 2026
-	4 th	5 th	1 st	7 th	3 rd	2 nd	5 th	3 rd	7 th	5 th	2 nd	7 th
CHAIR	AH	RF	TC	AH	RF	AH	TC	RF	TC	AH	RF	TC